

Major interactions

35 drug-drug findings graded major, grouped by mechanism. Severity grades the consequence; certainty is recorded separately as evidence. Suggested actions are in the records.

SEROTONERGIC AND CYP2D6 - THE SERTRALINE CLUSTER

Sertraline + Amphetamine IR Additive serotonergic activity, plus additive cardiovascular effect. Sertraline also weakly inhibits CYP2D6, a minor amphetamine route.	<i>additive toxicity</i>
Sertraline + Amphetamine XR Additive serotonergic activity, plus additive cardiovascular effect. Sertraline also weakly inhibits CYP2D6, a minor amphetamine route.	<i>additive toxicity</i>
Sertraline + Aspirin SSRIs deplete platelet serotonin and impair platelet aggregation; NSAIDs damage the mucosa and inhibit platelet COX-1. Combined, upper GI bleeding risk rises several-fold above either alone. One of the most under-recognised pairings in ordinary outpatient use.	<i>additive toxicity</i>
Sertraline + Dextromethorphan The clearest serotonin syndrome risk in this database, and it is a double hit: sertraline inhibits CYP2D6, which is dextromethorphan's main clearance route, so parent levels rise several-fold at the same time as serotonergic tone rises. Cases are well documented at ordinary OTC doses.	<i>additive toxicity</i>
Sertraline + Dextromethorphan CYP2D6 inhibition by sertraline converts an extensive metaboliser into a functional poor metaboliser. Recorded separately from the pharmacodynamic entry because the two need different mitigations: one is solved by dose, the other is not.	<i>metabolism</i>
Sertraline + Diclofenac SSRIs deplete platelet serotonin and impair platelet aggregation; NSAIDs damage the mucosa and inhibit platelet COX-1. Combined, upper GI bleeding risk rises several-fold above either alone. One of the most under-recognised pairings in ordinary outpatient use.	<i>additive toxicity</i>
Sertraline + Ibuprofen SSRIs deplete platelet serotonin and impair platelet aggregation; NSAIDs damage the mucosa and inhibit platelet COX-1. Combined, upper GI bleeding risk rises several-fold above either alone. One of the most under-recognised pairings in ordinary outpatient use.	<i>additive toxicity</i>
Sertraline + Naproxen SSRIs deplete platelet serotonin and impair platelet aggregation; NSAIDs damage the mucosa and inhibit platelet COX-1. Combined, upper GI bleeding risk rises several-fold above either alone. One of the most under-recognised pairings in ordinary outpatient use.	<i>additive toxicity</i>
Sertraline + Propranolol Moderate CYP2D6 inhibition by sertraline against a high-extraction CYP2D6 substrate. The effect scales with sertraline dose — slight at 50 mg, substantial at 200 mg — which means the interaction can appear weeks after both drugs were started.	<i>metabolism</i>

NSAID STACKING AND BLEEDING RISK

Diclofenac + Aspirin Additive gastrointestinal, renal and cardiovascular toxicity with no additive analgesic benefit — the COX enzyme is already saturated by one agent at therapeutic dose. GI bleeding risk rises multiplicatively.	<i>additive toxicity</i>
Ibuprofen + Aspirin Additive gastrointestinal, renal and cardiovascular toxicity with no additive analgesic benefit — the COX enzyme is already saturated by one agent at therapeutic dose. GI bleeding risk rises multiplicatively.	<i>additive toxicity</i>